

**EQUITABLE ACCESS TO
PHARMACEUTICALS SHOULD BE
PRIORITIZED OVER PROTECTING
INTELLECTUAL PROPERTY RIGHTS.**

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On the topic:

“Resolved: Equitable access to pharmaceuticals should be prioritized over protecting intellectual property rights.”

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INTRODUCTION

In an era where medical breakthroughs are more frequent than ever, the problem of accessing essential pharmaceuticals remains a critical issue that demands urgent attention. Intellectual property rights have often been defended as a requirement for innovation, but this has often been at an increased cost to accessing life-saving drugs. Millions of people, especially the low and middle-income countries, are condemned to suffer or lose their lives to diseases that could have been prevented because they cannot afford the high costs associated with patented drugs.

Shifting to the fair access of pharmaceuticals is not only a moral issue but it also remains an important key for equity in health and social justice in the world. Locking life-saving treatments behind paywalls of intellectual property laws seems to place profits over people. But this approach only widens health gaps further and suppresses that possibility of true collective well-being. By ensuring that everyone has access to necessary medications, we can foster healthier communities, reduce the burden on healthcare systems, and ultimately save lives.

DEFINING KEY TERMS

According to the World Health Organization (WHO), pharmaceuticals, also referred to as medicines or drugs, are specialized products used in both modern and traditional medicine. They serve essential functions in preventing, treating, and managing diseases while safeguarding public health whereas equitable access to pharmaceuticals, as defined by the World Health Organization (WHO), is the principle that all individuals should have fair and non-discriminatory access to necessary medicines. This concept encompasses several key dimensions that aim to ensure that medicines are available, affordable, and accessible to everyone, particularly vulnerable populations.

Also, when we talk about intellectual property rights, they refer to rights that protect the legal interests of creators by granting them exclusive control over their innovations and creations. These rights apply to various outputs of the mind, such as inventions, literary works, designs, and trademarks. The goal of IP rights is to strike a balance between encouraging innovation and benefiting society by ensuring creators earn recognition and economic rewards while contributing to public well-being. The WHO emphasizes that public health must take precedence when balancing access to essential medicines and intellectual property rights and in collaboration with the World Intellectual Property Organization (WIPO) and the World Trade Organization (WTO), the WHO underscores that intellectual property rules should not become barriers to accessing lifesaving medicines.

PUBLIC HEALTH SHOULD TAKE PRECEDENCE OVER PROFITS

Public health must come before profits, especially when lives are at stake. The need to prioritize public health, therefore, is more than a moral imperative but one necessitated by equal access to lifesaving medications for deadly diseases like HIV, malaria, and COVID-19. The experience of anti-HIV/AIDS medications stands out as one of the most striking examples of how prioritization of access over profits can be translated into significant health gains in the 1990s.

In the 1990s, the prices of antiretroviral drugs to treat HIV/AIDS were very high due to patent protection by pharmaceutical companies. For many developing countries, the yearly treatment cost could go up to 100 times the average gross domestic product per capita, thereby shutting these life-saving medications out of the reach of those who needed them. This led to sky-rocketing mortality rates in the worst-hit regions, particularly in sub-Saharan Africa. In the year 2000, a United Nations report estimated that up to half of all African teenagers could die of AIDS in their lifetime due to lack of treatment.

All this dramatically changed with the arrival of generic versions of these drugs. Once patents were loosened and generics became available, prices plunged, enabling millions of people in developing countries to access treatments they desperately needed. A key example is that Brazil's experience with generic AIDS drugs has shown how treatment costs plummeted, enabling health systems to treat thousands more patients at only a fraction of the previous expense. By 2007, deaths related to AIDS began to decline as access to these life-saving medicines improved.

This example proves that public health always comes first when governments put people's health first. When patent protection prevails, as in the first years of the HIV/AIDS crisis, access to treatment is severely restricted, thus worsening the public health emergency. On the other hand, when compulsory licensing, parallel imports, and similar access-enhancing policies are pushed and forced by governments and groups, health outcomes almost miraculously appear improved. The fact that much of the funding for HIV research came from public sources rather than private investments undercuts claims about patent protection being necessary as an incentive for researching drugs.

The same paradigm goes beyond HIV/AIDS and extends to other key health crises, like COVID-19. The pandemic has instigated calls for equitable access around the world to

vaccines and treatments. This idea is encapsulated in the call for a “people’s vaccine,” insisting that this lifesaving medication be treated as a public good, rather than being commodified and thus subject to market whims. The pegging of the debate around vaccine distribution during the pandemic brings into stark focus the view that saving life requires profits should give way to public health.

In short, placing public health over profit is not an ethical stance but a pragmatic requirement that could save millions of lives. The legacy of HIV/AIDS medications clearly demonstrates that when access is prioritized—through generics and reduced costs—outcomes in health drastically improve. As we forge ahead with prevailing global health difficulties, this principle should be leading the way into our health care policy and pharmaceutical practices.

MONOPOLIES INFLATE PRICES AND LIMIT ACCESS

The high prices inflate through the monopolies in the pharmaceutical sector, significantly based on patents, allowing access only to a few that can afford because most people, especially in poor countries, cannot afford these high prices. In cases where a company enjoys exclusive rights to produce a certain medication, it may be able to set prices so high that most of the people for whom such medication is particularly relevant cannot afford it. It becomes a blunt situation where an individual's ability to pay overshadows the need for treatment, with grave public health consequences.

For instance, the prices for patented medicines can be fantastically high in most developing countries, even as high as 100 times the average income. This is particularly true for the class of HIV/AIDS drugs in the late 1990s, where high prices prevented millions of people from receiving life-saving treatments due to patent monopolies. What really made things worse was that a majority of people with HIV lived in poorer countries which could ill afford them within the limited healthcare budgets.

For example, compulsory licensing in countries like India has shown how opening up production to generic versions of the drugs can drastically cut down prices. In one such case, after certain critical medicines were put under compulsory licensing, prices dropped as high as 97% for certain drugs like Sorafenib. This allowed wider access to treatment and better health outcomes for many patients who could never afford their medications.

Monopolistic practices result in an unjustified inaccessibility, especially for the most vulnerable. This may suggest that when pharmaceutical companies prioritize profits over public health, they support a system where only those who can pay exorbitantly high prices for treatment can access the necessary treatments. This dynamic not only exacerbates health inequities but also undermines global public health efforts.

Moreover, monopolies can suppress innovation rather than promote it. This is often offset by the fact that many life-saving drugs have been developed through significant public funding. Indeed, research into AIDS drugs, for example, revealed that much of the development was in fact paid for by taxpayers, not private investment. Thus, when patents are used mostly as an instrument of profit maximization rather than genuine innovation, they form a barrier to rather than assistance in the improvement of public health.

The final conclusion is that these patent-related monopolies inflate drug prices, preventing access to life-saving medicines for those who need them most. Lessons learned from those countries making use of compulsory licensing show that generic production ensures a reduction in prices and improvement in health outcomes. A system that puts profit before access will never succeed in responding to the acute health needs of the most vulnerable citizens and strengthens the rationale for changing policies to support public health at the expense of corporate profits.

ECONOMIC CONSIDERATIONS

The economic implications of pharmaceutical access versus intellectual property (IP) protection reveal that prioritizing affordable access to medicines is crucial for both public health and economic development, especially in low and middle-income countries (LMICs).

Pharmaceutical companies claim high drug prices are necessary to recover R&D costs, which can range from \$1.3 billion to \$2.6 billion per drug. However, much of this includes marketing expenses rather than pure research. Furthermore, many breakthroughs are publicly funded, as seen with COVID-19 vaccines, yet companies still sold these vaccines at premium prices, despite public contributions. Patents create monopolies that give companies exclusive control, resulting in inflated prices and limited affordability for patients and healthcare systems. In LMICs, where healthcare budgets are already strained, these high drug prices pose a severe challenge. Governments often face a difficult choice: either reduce access to essential medicines or divert funds from other critical areas.

Additionally, many people in these countries pay out of pocket for healthcare, driving families into poverty. The HIV/AIDS crisis is a clear example: in the 1990s, antiretroviral (ARV) treatment cost \$10,000 per person per year, making it inaccessible to millions. However, after generic production was allowed, the cost dropped by 90%, to about \$75 per person annually, saving countless lives.

Access to medicines directly impacts workforce health and economic productivity. In regions where infectious diseases like malaria or HIV are prevalent, untreated illnesses lead to lost labor and reduced productivity. For example, effective malaria treatment helps workers recover quickly, preventing income loss and maintaining productivity. Similarly, anti-retroviral therapy (ART) has allowed HIV-positive individuals to remain active in the workforce for many years, contributing to their economies. Healthcare access is not just a social issue but an economic investment. Countries that provide affordable medicines build economic resilience by preventing chronic illnesses, reducing healthcare costs, and ensuring higher workforce participation. Healthier individuals also support dependents and pay taxes, creating a ripple effect that boosts national development.

Conversely, untreated illnesses increase dependency rates and place additional pressure on social services, dragging down economic growth. In conclusion, high drug prices fueled by IP protections disproportionately harm LMICs, driving poverty and limiting economic

productivity. Prioritizing affordable access to medicines ensures not only better public health outcomes but also enhances national prosperity through a healthier, more productive workforce. Generics and compulsory licensing offer practical ways to balance innovation and access, ensuring everyone benefits from medical advances.

HEALTH CRISES DEMAND GLOBAL COLLABORATION

Quick access to lifesaving medications and vaccines is everybody's dire need in times of a global health crisis and it has been time for a collaborative approach transcending national boundaries and corporate interests. The COVID-19 pandemic dramatically proved that licensing for manufacturing and distributing an essential drug with one company cannot be sufficient but instead calls for multiple producers to meet the global demand in the shortest time and most effective way.

The scale-up of COVID-19 vaccines is an important example of where global collaboration can be deepened vis-à-vis public health outcomes. Various initiatives like COVAX, created by the WHO, Gavi, and the Coalition for Epidemic Preparedness Innovations or CEPI, were set up to ensure vaccine access for all countries on a principle of equality, with a focus on low and middle-income countries. COVAX was designed to accelerate the process of vaccine development and facilitate access to vaccines across populations in need, irrespective of their economic status. In aggregating resources, expertise, and manufacturing capabilities, COVAX has delivered more than 1.1 billion vaccine doses to countries around the world, though falling short of the stated target due to supply chain constraints and vaccine nationalism.

The capability of allowing wide production in times of health crises is very important. Patents and monopolistic practices in the bottlenecks of producing life-saving drugs delay treatments and prolong public health emergencies when supply cannot meet demand. During the early stages of the pandemic, for example, there was significant delay in vaccine access in many countries because few manufacturers were producing the vaccines. It clearly showed that the situation needed several firms to engage in the production of the vaccine so that the unprecedented demand on all levels could be curbed.

Moreover, international collaboration increases knowledge sharing and accelerates research. Working in unison means countries are able to share data regarding treatment options, come forward, and share experiences on the best practices of deploying vaccines and leveraging combined know-how for better responses to health crises. During COVID-19, the WHO solidarity trial brought more than 30 countries together in search of treatments against SARS-CoV-2. It presented a perfect example of how, from that form of collaboration, faster discoveries and better outcomes can result.

Further, beyond better access and improved timelines of production, global collaboration meets the ethical imperative for equity in healthcare. In a crisis where lives are at risk, all populations need access to the treatments needed. Especially in vulnerable populations, when companies put profits over the public good through strict enforcement of patents, barriers go up that hurt the low-income communities particularly. This dynamic underlines the need for policies that put interests in public health above those of corporations.

The bottom line of the COVID-19 pandemic is that health crises demand one response: a unified response by the global community. By allowing the wide production of essential medications, lives will be saved, and global health systems will be built with resilience against future pandemics. Moving forward, it will be important for governments and organizations to learn the value of collaboration in healthcare to ensure that, when the next crisis occurs, we can act faster and more equitably for all populations.

Lessons learned from the COVID-19 pandemic underpin the truism that global collaboration is not only desirable but an urgent necessity in responding effectively to health emergencies. We would then focus on widespread production and equitable access over monopolistic practices, allowing life-saving medications to reach those who need them most, to save lives and enhance global public health resilience.

CONCLUSION

It is not just about equity, but also of sheer public health necessity. We cannot afford to live in a world where millions succumb to diseases that are preventable or curable simply because we have prioritized IP rights over human lives. Such policy is not only immoral but also no less than a stumbling block on the road to achieving health-related Millennium Development Goals. These inequalities point to some of the most disturbing realities, which are solemnly showing the need for an immediate paradigm shift in our approach to pharmaceutical distribution and innovation.

The patent protection permission to dictate the access of lifesaving drugs has ethical connotations that are deep. In this case, where lifesaving drugs are outside the affordability of those whom it should be targeting, human life indirectly has a price. This approach disproportionately hits the more vulnerable populations, especially in low-income countries, whose health budgets are already overstretched. The repercussions, owing to such inequity, are dire: preventable deaths or the aggravation of health crises that could otherwise have been managed through timely access to medications.

Fortunately, there are alternatives that could work in balancing the need for innovation with imperatives for access. Public-private partnerships, government funding initiatives, and open-source models show paths toward incentivizing research and development while making lifesaving drugs reach all people irrespective of their ability to pay for them. These approaches indeed facilitate innovation while giving importance to public health outcomes and essentially create a more level playing field in the healthcare sector.

This is the delicate balance that global health policy must achieve as it moves into the future. The bottom line in population health needs to be recognized and understood as being far superior to the bottom line of corporate profits. This calls for governments, international organizations, and the companies themselves to take the responsibility of ensuring lifesaving drugs are available to people who need them, not just those with the ability to afford them. We can build a world in which health is considered a right by instituting policies that emphasize equitable access.

In short, the call for equal access to pharmaceuticals is not a moral appeal, per se, but part of the travel toward global health equity. It is about time that stakeholders at all levels commit themselves to policies which place human lives at the center of decisions on pharmaceuticals,

ensuring innovations serve the greater good rather than only corporate interests. This way, we can all help create a better future.

WORKS CITED

- “Effect of Drug Patents in Developing Countries.”* NCBI, National Center for Biotechnology Information, <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC1118638/>.
- “Generic HIV Medicines: Boon or Bust?”* Infectious Disease Advisor, <https://www.infectiousdiseaseadvisor.com/features/generic-hiv-medicines-boon-or-bust/>.
- “People over Profits: Make COVID-19 Medicines and Vaccines Free and Fair for All.”* Oxfam International, <https://www.oxfam.org/en/blogs/people-over-profits-make-covid-19-medicines-and-vaccines-free-and-fair-all>.
- “People over Profit: A Critical Paradigm Shift to Realize Health for All.”* PLOS Blogs, <https://speakinofmedicine.plos.org/2023/04/07/people-over-profit-a-critical-paradigm-shift-to-realize-health-for-all/>.
- “The Morality of Patenting Life-Saving Drugs.”* LSE Impact Blog, London School of Economics, <https://blogs.lse.ac.uk/impactofsocialsciences/2014/08/06/the-morality-of-patenting-life-saving-drugs/>.
- “Experts Call for Bold Action, Prioritization of Health over Profit in New Lancet Series.”* University of Melbourne, <https://www.unimelb.edu.au/newsroom/news/2023/march/experts-call-for-bold-action,-prioritisation-of-health-over-profit-in-new-lancet-series>.
- “Drug Price Controls in the United States.”* Congressional Research Service, <https://www.everycrsreport.com/reports/RL31066.html>.
- “Generic Antiretroviral Drugs for HIV Treatment: A Review.”* Wiley Online Library, <https://onlinelibrary.wiley.com/doi/10.1186/1758-2652-14-15>.
- “What Is the Impact of Intellectual Property Rules on Access to Medicines? A Systematic Review.”* Globalization and Health, vol. 18, Article 40, 2022, <https://globalizationandhealth.biomedcentral.com/articles/10.1186/s12992-022-00826-4>.
- “Drug Prices and Monopoly.”* Open Markets Institute, <https://www.openmarketsinstitute.org/learn/drug-prices-monopoly>.
- “USAID COVID Global Vaccine Access Capstone Report.”* USAID, September 2023, https://www.usaid.gov/sites/default/files/2023-09/USAID_COVID_GVAX_Capstone_Report-082423_FINAL_508.pdf.

“COVID-19 Vaccine Access and Allocation.” Taylor & Francis Online,
<https://www.tandfonline.com/doi/full/10.1080/13696998.2023.2207423>.

“Coronavirus Disease (COVID-19) Vaccine Access and Allocation.” World Health Organization, [https://www.who.int/news-room/questions-and-answers/item/coronavirus-disease-\(covid-19\)-vaccine-access-and-allocation](https://www.who.int/news-room/questions-and-answers/item/coronavirus-disease-(covid-19)-vaccine-access-and-allocation).

“COVID-19 Vaccines: A Review of Current Research.” PMC, National Center for Biotechnology Information, <https://pmc.ncbi.nlm.nih.gov/articles/PMC9862832/>.